

Losing Lyumjev: changes since publication

Each change below gives where it goes in the live post and the text as it reads. The WordPress block HTML for each change, in the same order and headed by the same instruction, is in `changes_since_publication_wordpress.html`; paste a block into the code editor (Options, Code editor) at the place given. The full regenerated post is `diabettech-ultra-rapid-article-wordpress.html` in the same folder, if replacing the whole body is easier. Generated by `additions_since_publication.py` from the master article, compared with the live post as saved on 2026-10-03. New figures are marked as placeholders; the image files are in the Drive folder Diabettech article/figures.

1. Replacement paragraph

Where: Replace the paragraph "The UK has had not seen any announcement."... with the text below. It comes straight after the paragraph ending "specialist prescribing had been very low."

Text:

The UK has not seen any announcement. The NHS medicines dictionary marks only the Lyumjev Tempo Pen as discontinued, and Lilly's March notice covers the EU and EEA. The UK's share of Lyumjev is the same as France's, which has already lost the cartridges, and Lilly has not said what it plans here.

2. Replacement paragraph

Where: Replace the paragraph "The UK was always going to struggle with"... with the text below. It comes straight after the paragraph ending "clinicians don't know enough about it."

Text:

The UK was always going to struggle with this, as by far the most frequently prescribed rapid acting insulin comes from Novo (even though pricing for Novo and Lilly products has historically been similar) so it would have required a major marketing push to get Lyumjev in front of prescribers. In the year to July 2026, Lyumjev was 3.7% of the rapid-acting analogue insulin units dispensed in English primary care, six years after it arrived. Its share of lispro has grown every year, to 24% in 2026. Fiasp adds 13.9%, so the two ultra-rapid insulins together are 17.5% of units. It's notable that Fiasp is still a larger share than Humalog. Other European markets split differently. In France in 2025 Humalog was 27.3% of rapid-acting analogue units and Fiasp 11.1%; in Denmark, Novo Nordisk's home market, Fiasp was 11.0% and Humalog 1.1%. The other 82.5%, or 2.74 million prescription items a year, is one of the older products.

3. New figure

Where: Insert after the paragraph ending "almost everywhere, is a written place." and before the paragraph "Part of the difference is process. As a".

Text:

[FIGURE PLACEHOLDER: upload fig5_tresiba_vs_lyumjev_formulary.png from the Drive folder Diabettech article/figures. Caption: Formulary position of Lyumjev and Tresiba in 38 UK formulary areas, read in September and October 2026. Each area is counted once for each insulin.]

4. New paragraph with figure

Where: Insert after the paragraph ending "showing that they caused the rise." and before the heading "Cost before biosimilars".

Text:

Abroad the route differs, and appraisal can shut a door as well as open one. France's assessment committee re-rated Tresiba in 2019 as a minor improvement over glargine (ASMR IV) for people at high risk of hypoglycaemia, on the strength of those trials, while it rated [Fiasp](#) and [Lyumjev](#) as no improvement (ASMR V). In Germany every benefit assessment of degludec found no added benefit, and [Novo Nordisk took Tresiba off the market](#) from January 2016 until December 2018. Australia's [PBAC rejected it](#) in 2013, and it was subsidised only from July 2026, while Fiasp was listed there by the end of 2019 at the same price as NovoRapid. Outside Australia, wherever both can be measured, Tresiba now takes a larger share of long-acting insulin than the ultra-rapid insulins take of rapid-acting: 22% against 15% in France in 2025, 41% against 11% in Denmark, and 20% against 9% in the US Medicare drug programme in 2024 ([figures](#)). Price parity for the faster insulins was also a mostly British arrangement. In France Fiasp is listed at €2.09 per 100 units against €1.48 for NovoRapid, and in Denmark at 32.1 kroner against 16.7. Denmark's reimbursement committee counted Fiasp among "de dyrere lægemidler" (the more expensive medicines) in 2019, and since 2022 it has been reimbursed only for people whose after-meal glucose is not managed on another rapid insulin.

[FIGURE PLACEHOLDER: upload fig6_degludec_vs_ultra_countries.png from the Drive folder Diabettech article/figures. Caption: Degludec (Tresiba) as a share of long-acting analogue insulin against the ultra-rapid share of rapid-acting analogue insulin, latest year available. Germany's ultra-rapid share is a range because its data pool Lyumjev with Liprolog; Australia counts PBS prescriptions, and degludec was not subsidised there until July 2026.]

5. Replacement paragraph

Where: Replace the paragraph "For much of the period, the price of"... with the text below. It comes straight after the heading "Cost before biosimilars".

Text:

For much of the period, the price of the insulin offers little explanation. Lyumjev has cost English primary care the same per unit as Humalog in every year since it arrived. Its list price per box is the same as Humalog's. Fiasp was the same against NovoRapid. For the 52 months between Fiasp's arrival in February 2017 and the first biosimilar, Trurapi, in June 2021, English primary care paid £1.68 per 100 units of Fiasp against £1.76 for NovoRapid. There was little or no acquisition-cost premium for choosing the faster formulation. That is not the same as no cost to the system: switching takes clinic time and training, and pumps and pens have to be compatible. Those costs are not measured here.

6. New paragraphs with figure, then new subsection "Where the savings should come from"

Where: Insert after the paragraph ending "Lyumjev, after six, is at 3.7%." and before the heading "Lilly's part".

Text:

Outside England, biosimilars don't explain the low use of the faster insulins. In Denmark and Germany they were about 5% of rapid-acting analogue insulin in 2025, and in France 2.9%. The discount on NovoRapid in France and Denmark is only 4 to 5%, and in each country the faster insulins held at 11% to 15%. In the US Medicare drug programme Admelog costs the same as Humalog. England is where that could change. Trurapi lists at 21% less than NovoRapid overall and 30% less for pens. Its share of rapid-acting units rose from 0.3% in 2022 to 6.0% in 2025, pushed by incentive schemes such as Dorset's and by formularies such as North West London's that send every new start to it. Within the UK, the two nations with the most ultra-rapid use, Wales at 30.9% and Northern Ireland at 23.9%, dispensed almost no biosimilar: 2.1% in Wales and none at all in Northern Ireland. The concern is that once England makes the cheaper standard-speed insulin the default for new starters, the people who might have done better on a faster insulin are never offered one, and the gap with Wales grows ([figures](#)).

Where the savings should come from

The biosimilar effort has been pointed at the wrong insulin. In the year to July 2026, at list price and device for device, Fiasp cost English primary care exactly what NovoRapid cost, and Lyumjev exactly what Humalog cost. Moving someone from either older insulin to its faster version costs nothing. The only money at stake is the saving given up by not moving them to Trurapi, which is £0.47 per 100 units cheaper than Fiasp. Lispro has no cheaper alternative left to protect, since Sanofi is withdrawing Admelog.

Long-acting insulin is where a switch asks nobody to give anything up. Semglee, the cheapest glargine, lists at £2.00 per 100 units against £2.32 for Lantus and £2.35 for Abasaglar, the older biosimilar, and it is the same molecule with the same profile of action. Yet it is 4.8% of the glargine 100 dispensed in England. Moving 80% of Lantus and Abasaglar pens to it would save about £5.5 million a year. That is enough to pay for 14.5% of all the rapid-acting insulin dispensed in England to be Fiasp in place of Trurapi, and close to the £5.8 million, at most, that bringing England up to Wales's ultra-rapid share would forgo ([figures](#)).

North West London shows the blunt version. Its formulary says "All new initiations should be the lower cost biosimilar, Trurapi, instead of NovoRapid". It leaves Fiasp off and makes Lyumjev a second-line choice for specialists to start. For long-acting insulin it lists Lantus, Abasaglar and Semglee side by side, with no preference for the cheapest. Its ultra-rapid share is 10.1%, the 12th lowest of 106 areas in England. Even the biosimilar push has gone slowly there: Trurapi is 9.2% of its standard aspart and Semglee 5.1% of its glargine. Moving 80% of its glargine pens to Semglee would save about £300,000 a year, more than the £270,000 it would give up in reaching Wales's ultra-rapid share. A more granular policy, pressing the biosimilar where speed is not at stake and leaving the faster mealtime insulins open to anyone who wants them, would have saved money and widened choice at the same time.

[FIGURE PLACEHOLDER: upload fig7_biosimilar_savings.png from the Drive folder Diabettech article/figures. Caption: Annual figures at list price, August 2025 to July 2026. The first two bars are the savings from moving 80% of Lantus and Abasaglar pens to Semglee and 80% of NovoRapid to Trurapi. The third is the most it would cost, in Trurapi savings given up, to reach Wales's ultra-rapid share.]

These are list prices, which leave out confidential discounts, and they keep everyone on the same kind of device. Semglee is sold only as a pen, so the 230 million units of Lantus and Abasaglar in cartridges are left out.

7. Replacement paragraph

Where: Replace the paragraph "These are what payers spent at list or"... with the text below. It comes straight after the heading "What "commercial reasons" means in money".

Text:

These are what payers spent at list or reimbursement prices, and they give a sense of scale. In England, primary care spent £5.6 million on Lyumjev in the year to July 2026, against £18.3 million on Humalog. In France it was €6.4 million against €49.3 million. In the US Medicare drug programme Lyumjev took \$90 million in 2024, against \$664 million for Humalog. Lyumjev is between a tenth and a quarter of what payers spent on Lilly's rapid-acting insulins in each of these markets ([figures](#)).

8. Replacement paragraphs

Where: Replace the 2 paragraphs from the paragraph "What I'd like to see from here is"... to the paragraph "Is it a failure of the system? Yes,"... with the text below. It comes straight after the paragraph ending "one health board more than half".

Text:

What I want to see now is the diabetes charities and patient organisations of Europe standing up and being counted, and so far they have barely made a sound. Diabetes UK's page on insulin supply lists ten discontinuations and shortages and does not mention Lyumjev or the withdrawals across Europe, and Breakthrough T1D UK's site mentions Lyumjev only in its guide to connected pens. In France, which has already lost the cartridges, the last thing I can find on Lyumjev from the national federation of people with diabetes is a 2023 note about a temporary shortage. Germany's diabetesDE has nothing on it, and the German Diabetes Society's statement was practical advice on pump cartridge shortages ([search record](#)). Norway's diabetes association reported the withdrawals and quoted a professor reassuring readers that there were good alternatives to every insulin going. diabetesschweiz complained to Lilly, then passed on Lilly's explanation that hardly anyone used it. Reporting a withdrawal and relaying the manufacturer's reasons is the least an organisation can do, and these organisations exist to speak for the people who use these insulins. They should be demanding that the faster insulins stay on the market, that they are part of the fully closed-loop systems now in trials, that formularies stop parking them behind the insulin people are already on, and that clinicians offer them to everyone who might benefit and let each person decide for themselves whether to turn them down. When they stay quiet, the manufacturer's account of low demand is the only one anyone hears.

Is this a failure of the system? Yes, at every level, and nobody escapes the blame. The licensing trials were built to show the new insulins were no worse on HbA1c, and that was read as showing they were no better. NICE left its mealtime insulin advice as it stood in 2015, looked at it again in January 2026 and left it there. The Scottish and Welsh appraisal bodies wrote rules that left Lyumjev without a decision in either country, and Northern Ireland, which depends on them, inherited the gap. Formulary committees left it off or put it behind the insulin people were already using, while finding a written place for Tresiba, which costs a third more than Lantus. Consultants told people who asked for a faster insulin that they were too well controlled to need one. Lilly priced Lyumjev the same as Humalog and then, as far as the public record shows, did almost nothing to make the case for it in the UK, and did not even tell the Swiss patient organisation that it was going. The charities, whose whole purpose is to make noise about exactly this, have barely raised their voices. The cost of all of it falls on the people with diabetes who were never offered the choice.

9. New sources 61 to 70

Where: Add at the end of the numbered Sources list, after entry 60.

Text:

1. Haute Autorité de Santé, Commission de la Transparence. Tresiba, avis CT-17265 (re-evaluation, ASMR IV in adults at high risk of hypoglycaemia), 6 February 2019; Fiasp, avis CT-16059 (ASMR V), 19 July 2017; Lyumjev, avis CT-18474 (ASMR V), 10 June 2020. https://www.has-sante.fr/jcms/c_2906439 ; https://www.has-sante.fr/jcms/c_2788573 ; https://www.has-sante.fr/jcms/p_3190312
2. Gemeinsamer Bundesausschuss. Insulin degludec, Nutzenbewertung nach § 35a SGB V, procedures 109, 121, 162, 168 and 415 (2014 to 2019), each "Ein Zusatznutzen ist nicht belegt"; Deutsche Apotheker Zeitung, Endgültiges Aus für Tresiba, 16 December 2015; *arznei-telegramm* 2019;50:17 on the return from December 2018. <https://www.g-ba.de/bewertungsverfahren/nutzenbewertung/415/> ; <https://www.deutsche-apotheker-zeitung.de/news/artikel/2015/12/16/endgueltiges-aus-fur-tresiba>
3. Pharmaceutical Benefits Advisory Committee. Insulin degludec, Public Summary Document, March 2013 and July 2025; Fiasp, Public Summary Document, November 2018; PBS Medicine Status, insulin degludec, listed 1 July 2026. <https://www.pbs.gov.au/industry/pbac/psd/2013/03/insulin-psd-032013.pdf> ; <https://www.pbs.gov.au/industry/pbac/psd/2018/11/insulin-aspart-psd-november-2018.pdf> ; <https://www.pbs.gov.au/medicinesstatus/document/1406>
4. Lægemiddelstyrelsen. Ny tilskudsstatus for insuliner, decision of 14 March 2022 (Fiasp: generelt klausuleret tilskud); Tresiba får generelt tilskud, 16 April 2026; *medicinpriser.dk* prices read 2 October 2026. <https://www.medicinpriser.dk>
5. Long-acting insulin by country, prices and appraisal records for degludec, glargine 300, Fiasp and Lyumjev in France, Denmark, Norway, Germany, Australia and the US: <https://github.com/tim2000s/rapid-insulin-prescribing/tree/master/international/scoping> (each country's basal folder); combined in *international/output/BASAL_INTL.md*
6. Deutsche Diabetes Gesellschaft. Stellungnahme DDG zu möglichen Engpässen der Insuline NovoRapid PumpCart und Fiasp PumpCart sowie anderer Insuline. July 2025. <https://www.ddg.info/politik/stellungnahmen/stellungnahme-ddg-zu-moeglichen-engpaessen-der-insuline-novorapid-pumpcartr-und-fiasppumpcartr-sowie-anderer-insuline>

7. Diabetes UK. What you need to know about insulin supply issues in the UK, updated 14 November 2025, read 3 October 2026 (including the Medicine Supply Notification of 18 September 2025 for the discontinuation of Admelog); and the searches of Breakthrough T1D UK, Fédération Française des Diabétiques, diabetesDE, diabetesschweiz and the Belgian associations recorded at https://github.com/tim2000s/rapid-insulin-prescribing/blob/master/withdrawals_eu/charities/SEARCH_LOG.md
8. North West London Integrated Formulary, version 64, last updated 3 September 2026, section 6.1.1 (insulins). Captured in https://github.com/tim2000s/rapid-insulin-prescribing/tree/master/formulary_basal/raw (nwl_integrated_formulary)
9. List prices by device and the biosimilar scenarios for England and North West London, August 2025 to July 2026: <https://github.com/tim2000s/rapid-insulin-prescribing/blob/master/output/biosimilar/SUMMARY.md>
10. Biosimilar rapid-acting insulin by country and year, with prices against the originator: https://github.com/tim2000s/rapid-insulin-prescribing/blob/master/international/output/RAPID_BIOSIMILAR_INTL.md